INDEPENDENT REGULATORY COMPLIANCE ASSESSMENT

HOSPITAL PRICE TRANSPARENCY AUDIT

Independent Diagnostic Gap Analysis & Regulatory Exposure Assessment (45 CFR Part 180)

DATE: October 24, 2026

STANDARD: Hospital Price Transparency (45 CFR 180)

AUDITED HEALTHCARE FACILITY

Bibb Medical Center

Target MRF Resource: 9927c3_bdf2750fd3e64fb4906ea7c7a5cfccfc.xlsx [->]

NON-COMPLIANT FORMAT: UNAUTHORIZED FILE TYPE

Statutory 45 CFR Section 180.50(c)(2) Violation

SCANNED ROWS	FLAGGED VIOLATIONS	SAFETY SCORE
0	1	0.0% (Grade F)
Machine-Readable CDM Records	File-Level Statutory Defect	Compliance Benchmark (>=90%)

EXECUTIVE DIAGNOSTIC SUMMARY : STATUTORY FINDINGS

An algorithmic compliance audit of the public Machine-Readable File (MRF) published by Bibb Medical Center identified an unauthorized file format under federal price transparency mandates (45 CFR Section 180.50(c)(2)). CMS strictly mandates standard charges in standardized CSV or JSON formats; proprietary spreadsheet (.xlsx, .xls) and document formats are prohibited. Because the published file could not be ingested due to this pre-ingestion format barrier, internal chargemaster rows were not audited.

Publishing standard charge data in an unauthorized format constitutes an explicit statutory non-compliance defect, leaving the facility vulnerable to automated CMS enforcement audits, mandatory Corrective Action Plans (CAP), and compounding daily civil monetary enforcement liabilities under 45 CFR Section 180.90.

CMS SCHEMA INTEGRITY	PAYER RATE COMPLETENESS	ENFORCEMENT LIABILITY
INVALID FORMAT	NOT AUDITED	ACTIVE CMP EXPOSURE
45 CFR § 180.50(c)(2)	Pre-Ingestion Barrier	45 CFR Section 180.90

PUBLIC AUDIT NOTICE: This independent technical diagnostic report evaluates publicly available hospital standard charge datasets pursuant to federal Hospital Price Transparency mandates (45 CFR Part 180).

SECTION I : REGULATORY RISK ASSESSMENT

Statutory Validation Failure & Enforcement Risk

Federal price transparency regulations (45 CFR Part 180) mandate public machine-readable standard charges. Publishing in unauthorized formats or non-compliant schemas halts programmatic ingestion and exposes the facility to statutory enforcement actions and compounding daily civil monetary penalties under 45 CFR Section 180.90.

STATUTORY CIVIL MONETARY PENALTIES (45 CFR SECTION 180.90)

The Centers for Medicare & Medicaid Services (CMS) strictly enforces MRF compliance through automated crawlers and targeted regulatory audits. Hospitals that fail to publish complete standard charge data face daily administrative fines scaled by licensed facility bed count:

Base Fine (<= 30 beds): $300 / day ($109,500 / year)

Scaled Fine (> 30 beds): $300 to $5,500 / day ($10 / bed / day) | Max: $2,007,500 / year

SECTION II : COMPLIANCE DEFECT LEDGER

Pre-Ingestion Barrier Audit

Because the published file could not be ingested due to an unauthorized format (such as .zip, .xlsx, or .xls), internal chargemaster rows were not audited. The statutory defect recorded below is strictly at the file level.

TOTAL DETECTED DEFECTS	DEFECT FREQUENCY RATIO
1	N/A
Statutory Defect	Pre-Ingestion Barrier (0 Ingested)

STATUTORY ENFORCEMENT & ADMINISTRATIVE ESCALATION TIMELINE (45 CFR SECTION 180.70 - 180.90)

CMS administrative enforcement protocols triggered upon non-compliance detection:

1. Written Warning Notice | 30-Day Statutory Opportunity to Cure

CMS issues formal written warning notice citing unauthorized file format non-compliance under 45 CFR Section 180.50(c)(2).

2. Mandatory Corrective Action Plan (CAP) | Formal Remediation Protocol

Hospital must submit a detailed CAP within 30 to 90 days outlining technical remediation steps, validation gates, and milestones.

3. Civil Monetary Penalty (CMP) & Registry Notice | Compounding Daily Fines

Accrual of daily fines scaled to facility capacity ($300 to $5,500/day, up to $2,007,500/year under 45 CFR Section 180.90), combined with permanent public listing on the official CMS Hospital Price Transparency Non-Compliance Registry.

SECTION III : SAMPLE VIOLATION AUDIT

Verified Statutory Format Record

Below is the verified statutory format non-compliance record identified for your published file. Because the file was published in an unauthorized format under 45 CFR Section 180.50(c)(2), internal chargemaster rows could not be parsed or audited.

ROW #	CPT / CODE	VIOLATION TYPE	DESCRIPTION & STATUTORY JUSTIFICATION
File Level Pre-Ingestion	**FORMAT** Scope	**Non-Compliant File Format**	**Facility published standard charges in unauthorized proprietary format '.xlsx'.** Under 45 CFR Section 180.50(c)(2), hospital standard charges must be published strictly in standardized CSV (.csv) or JSON (.json) schema formats.

PRE-INGESTION AUDIT STATUS: CHARGEMASTER ROWS NOT AUDITED

The compliance defect documented above represents a file-level statutory format barrier under 45 CFR Section 180.50(c)(2). Because the standard charges dataset was published in an unauthorized spreadsheet format rather than standardized CMS CSV or JSON templates, ingestion halted prior to scanning data rows.
Individual CPT/HCPCS procedure codes, negotiated commercial payer rates, and gross pricing amounts were NOT audited. Consequently, no line-by-line remediation error ledger was generated.

AUDIT COVERAGE	STATUTORY DEFECT	ROW-LEVEL AUDIT
0 Records Scanned	**Unauthorized Format**	**Not Audited**
Pre-Ingestion Barrier	45 CFR Section 180.50(c)(2)	Ingestion Gate Halted

Data Governance Notice: Pre-ingestion diagnostic audit findings are maintained under strict facility confidentiality pursuant to federal compliance standards.

CMS TEMPLATE STANDARDIZATION & SCHEMA SPECIFICATION CHECKLIST

Statutory technical architecture requirements under 45 CFR Section 180.50(c)(2) for standardized machine-readable files:

1. ROOT METADATA HEADERS

CMS Data Dictionary v3.0 Table 1

- Mandatory metadata headers (hospital_name, last_updated_on, version, affirmation, npi).
- Affirmation statement signed by an authorized hospital official under penalty of perjury.

2. STANDARD CHARGE DATA MODEL

CMS-1784-F & OPPS Mandates

- Gross charges, discounted cash prices, and payer-negotiated commercial rates.
- De-identified minimum and maximum negotiated rates across all standard charge items.

3. CLINICAL CODING TAXONOMY

CMS Table 2 Permitted Code Sets

- Canonical billing codes: CPT (5-digit), HCPCS, MS-DRG, APR-DRG, and NDC numbers.
- Zero proprietary CDM charge codes without national crosswalk mappings.

4. PAYLOAD FORMAT & INTEGRITY

45 CFR Section 180.50(c)(2) Standards

- Remediation of Unauthorized Format (.xlsx, .zip, disguised HTML) to standard CSV/JSON.
- Strict RFC 4180 CSV syntax or JSON Schema v3.0 validation without bot barriers.

SECTION IV : TECHNICAL REMEDIATION GUIDANCE

Statutory Format Remediation & CMS Schema Specifications

To resolve the statutory format defect identified in this diagnostic audit and achieve compliance under 45 CFR Section 180.50(c)(2), standard charges must be migrated to authorized CMS machine-readable templates. Official schema specifications and technical adoption guidance are outlined below.

STATUTORY NON-COMPLIANCE NOTICE (45 CFR § 180.50)

CMS Standardized Schema Migration Protocol

- Official CMS Machine-Readable File schema adoption guidance (CSV/JSON templates)
- Statutory non-compliance documentation citing 45 CFR Section 180.50(c)(2) format mandates
- Pre-ingestion barrier analysis detailing machine-readability conversion requirements
- Technical checklist for hospital IT & revenue cycle teams to eliminate CMP penalty exposure

TARGET FORMAT	AUDIT COVERAGE	STATUTORY AUTHORITY
Standardized CSV or JSON	**Pre-Ingestion Barrier (0 Rows Audited)**	**45 CFR Section 180.50(c)(2)**

CMS SCHEMA SPECIFICATIONS & TECHNICAL ADVISORY

Official CMS template standards, technical resources, and advisory contact:

Official CMS Repository: https://github.com/CMSgov/hospital-price-transparency

Authorized Layouts: Standardized CSV (Tall or Wide) | Standardized JSON (v3.0 Schema)

Direct Outreach: audit@elitedatasolution.net **Web Portal:** https://elitedatasolution.net

- **File Export:** Export chargemaster directly from hospital billing systems to standardized CSV.
- **Header Syntax:** Ensure file headers strictly follow CMS Data Dictionary v3.0 Table 1 requirements.
- **Crawler Access:** Verify published standard charges file is directly downloadable without bot defense barriers.